

1 ENGROSSED HOUSE
2 BILL NO. 4457

By: Newton and Deck of the
House

3 and

4 Jech of the Senate

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7 An Act relating to pharmacy benefits manager;
8 defining terms; prohibiting pharmacy benefits
9 managers from doing certain acts; requiring pharmacy
10 benefits managers to offer certain things; providing
11 for enforcement by the Attorney General; providing
12 for promulgation of rules; providing for
13 codification; and providing an effective date.

14 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

15 SECTION 1. NEW LAW A new section of law to be codified
16 in the Oklahoma Statutes as Section 6958.1 of Title 36, unless there
17 is created a duplication in numbering, reads as follows:

18 A. As used in this section:

19 1. "Health insurer" means any corporation, association, benefit
20 society, exchange, partnership or individual licensed by the
21 Oklahoma Insurance Code;

22 2. "Network pharmacy" means pharmacy providers contracted with
23 a pharmacy benefits manager;

24 3. "Pharmacy benefits manager" or "PBM" means a person,
business, or other entity that performs pharmacy benefits

1 management. The term shall include a person or entity acting on
2 behalf of a PBM in a contractual or employment relationship in the
3 performance of pharmacy benefits management for a managed care
4 company, nonprofit hospital, medical service organization, insurance
5 company, third-party payor or a health program administered by a
6 department of this state;

7 4. "Medically integrated pharmacy" means a pharmacy that:

- 8 a. is owned by, affiliated with, or under common
9 ownership with a specialty provider or specialty
10 provider practice,
- 11 b. is physically or operationally integrated within the
12 clinical practice of the specialty provider,
- 13 c. is licensed and in good standing with the Oklahoma
14 State Board of Pharmacy, and
- 15 d. employs at least one pharmacist licensed in this state
16 to dispense prescription medications;

17 5. "Specialty medication" means a prescription drug that:

- 18 a. is used to treat complex, chronic, or rare medical
19 conditions and requires specialized handling,
20 administration, monitoring, or patient management, or
- 21 b. is classified or designated as a specialty medication
22 by the pharmacy benefits manager or the health benefit
23 plan; and

1 6. "Specialty provider" means a licensed physician that
2 provides ongoing specialized medical treatment requiring complex
3 medication management, including but not limited to providers
4 practicing in the fields of oncology, urology, neurology,
5 nephrology, dermatology, cardiology, anesthesiology, or orthopedics.

6 B. A pharmacy benefits manager shall not:

7 1. Restrict, limit, or prohibit a covered person from obtaining
8 a specialty medication from a medically integrated pharmacy
9 affiliated with the covered person's treating specialty provider;
10 provided, that the pharmacy is licensed and in good standing with
11 the Oklahoma State Board of Pharmacy;

12 2. Require that a covered person obtain a specialty medication
13 exclusively through a pharmacy owned by, affiliated with, or
14 designated by the PBM when the covered person's treating specialty
15 provider maintains a medically integrated pharmacy capable of
16 dispensing the medication;

17 3. Discriminate against or disadvantage a medically integrated
18 pharmacy in the terms and conditions of network participation;

19 4. Impose additional or more stringent administrative
20 requirements on a medically integrated pharmacy that are not equally
21 imposed on other network pharmacies dispensing specialty
22 medications;

1 5. Deny, reduce, or delay reimbursement for a specialty
2 medication solely because the medication is dispensed by a medically
3 integrated pharmacy;

4 6. Require a specialty provider or covered person to transfer a
5 valid prescription to another pharmacy as a condition of coverage or
6 reimbursement;

7 7. Impose differential copayments, coinsurance, or other cost-
8 sharing on a covered person based solely on the pharmacy from which
9 the specialty medication is obtained;

10 8. Engage in patient steering or any practice that has the
11 effect of directing patients away from a medically integrated
12 pharmacy to a PBM-owned, -affiliated, or -preferred pharmacy; and

13 9. Implement any policy, prior authorization requirement, step
14 therapy protocol, or utilization management practice that has the
15 effect of preventing a specialty provider from dispensing
16 medications necessary to ensure continuity of care, reduce
17 unnecessary treatment delays.

18 C. A PBM shall offer a medically integrated pharmacy the same
19 opportunity to participate in specialty pharmacy networks as is
20 offered to any other pharmacy. Any reimbursement rate, fee
21 schedule, audit standard, credentialing requirement, or contractual
22 term applied by a PBM to a medically integrated pharmacy shall be no
23 less favorable than those applied to any PBM-owned, -affiliated, or
24 -preferred specialty pharmacy.

1 D. When a specialty provider is actively managing a covered
2 person's treatment plan, including monitoring for adverse effects,
3 drug interactions, or therapeutic outcomes, the PBM shall permit the
4 covered person to obtain specialty medications from the specialty
5 provider's medically integrated pharmacy. Nothing in this
6 subsection shall be construed to require a PBM to cover a drug that
7 is not otherwise covered by the health insurer.

8 E. A violation of this section shall constitute an unfair or
9 deceptive act and shall be subject to enforcement by the Attorney
10 General. The Attorney General shall promulgate rules as necessary
11 to implement and enforce the provisions of this section.

12 SECTION 2. This act shall become effective November 1, 2026.

13 Passed the House of Representatives the 23rd day of March, 2026.

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Presiding Officer of the House
of Representatives

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18 Passed the Senate the ____ day of _____, 2026.

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Presiding Officer of the Senate